BASHIR CLINIC & ALLERGY CENTER

RAG-Ready Clinic Knowledge Base

Version 1.0 | Effective: September 2026

IMPORTANT: This is a synthetic/demo knowledge base created for RAG testing. It is not verified real-world information about any actual clinic. Replace demo details with clinic-approved facts before production use.

Design goal: clear, self-contained sections for semantic chunking, multilingual retrieval, reranking, and strict evidence-based answers.

1. Document Purpose and Authority

This document is the static knowledge source for Bashir Clinic & Allergy Center in a hospital AI assistant. It is written so that each section can be retrieved independently by a RAG system. The assistant should use this document for clinic policies, services, preparation instructions, report timelines, and general clinic information.

Live operational information such as currently available appointment slots, appointment status, patient records, and schedule changes should come from the clinic database rather than this PDF.

Document status: Demo / synthetic test content
Version: 1.0
Effective date: 21 September 2026
Review cycle: Review every 90 days or whenever a clinic policy changes
Content owner: Clinic administration
Production rule: Clinic staff must verify every fact before replacing the demo label

2. Clinic Profile

Clinic name: Bashir Clinic & Allergy Center
Clinic type: Outpatient allergy, asthma, respiratory, and skin-allergy clinic
Demo address: 22 Clinic Road, Satellite Town, Rawalpindi, Punjab, Pakistan
Main reception: 051-000-0147
WhatsApp: 0300-000-0147
Email: care@bashirallergyclinic.example
Website: https://bashirallergyclinic.example
General clinic hours: Monday to Saturday, 9:00 AM to 8:00 PM
Sunday: Closed except for pre-arranged testing or urgent follow-up approved by the clinic
Public holidays: Hours may change; patients should confirm with reception before travelling

2.1 Accessibility and Facilities

- Wheelchair access is available at the main entrance.
- A wheelchair can be requested from reception.
- A waiting area is available for patients and one accompanying person.
- A prayer area is available inside the clinic building.
- Limited parking is available in front of the clinic; additional public parking is nearby.
- The clinic is smoke-free.
- Children should remain with a parent or guardian while inside the clinic.

3. Clinical Team and Consultation Scope

For production use, doctor availability and bookable slots should be read from the live scheduling database. The profiles below are static demo descriptions only.

3.1 Dr. Hamid Bashir - Consultant Allergist and Immunologist

Dr. Hamid Bashir evaluates allergic rhinitis, asthma, urticaria, eczema associated with allergy, food allergy, drug allergy, insect allergy, recurrent allergic symptoms, and patients being considered for allergen immunotherapy.

Demo consultation fee: Rs. 2,500

Typical consultation duration: 20 to 30 minutes

Demo clinic sessions: Monday to Saturday, 10:00 AM to 2:00 PM; Monday and Thursday, 5:00 PM to 8:00 PM

New-patient focus: Detailed allergy history, trigger review, examination, and test planning when indicated

3.2 Dr. Sana Bashir - Consultant Pulmonologist

Dr. Sana Bashir evaluates asthma, chronic cough, wheeze, breathing difficulty, allergic airway disease, spirometry results, and respiratory symptoms that may overlap with allergy.

Demo consultation fee: Rs. 2,200

Typical consultation duration: 20 minutes

Demo clinic sessions: Tuesday, Thursday, and Saturday, 3:00 PM to 6:00 PM

3.3 Dr. Areeba Khan - Dermatology and Allergy Consultant

Dr. Areeba Khan evaluates eczema, contact dermatitis, recurrent rashes, urticaria, suspected cosmetic or occupational allergy, and patients who may need patch testing.

Demo consultation fee: Rs. 2,000

Typical consultation duration: 20 minutes

Demo clinic sessions: Wednesday and Friday, 4:00 PM to 7:00 PM

4. Services Offered

Allergy Consultation

Evaluation of suspected environmental, food, medicine, insect, skin, or respiratory allergy. The consultation includes history, examination, and a plan for testing or treatment when clinically appropriate.

Asthma and Wheeze Assessment

Assessment of recurrent wheeze, cough, chest tightness, exercise-related symptoms, nighttime symptoms, and suspected allergic asthma. Spirometry may be recommended.

Allergic Rhinitis and Sinus Allergy

Evaluation of sneezing, blocked or runny nose, itchy nose, watery eyes, seasonal symptoms, dust-related symptoms, and suspected pollen or mite allergy.

Food Allergy Evaluation

Evaluation of reproducible symptoms after specific foods. Testing is selected according to clinical history. Broad food panels are not recommended without a clear clinical reason.

Drug Allergy Evaluation

Assessment of suspected reactions to medicines. A drug challenge is performed only when specifically planned by the clinician and when the clinic has confirmed that the procedure is suitable.

Skin Allergy and Urticaria

Evaluation of hives, itching, recurrent swelling, eczema, and suspected allergic skin disease.

Contact Allergy and Patch Testing

Evaluation of delayed skin reactions that may be caused by metals, cosmetics, fragrances, occupational products, adhesives, or other contact substances.

Allergen Immunotherapy

Long-term treatment for selected patients with confirmed clinically relevant allergy. It may be given by injection or another method depending on the clinician's plan and product availability.

Spirometry

A breathing test used to measure airflow and lung function. It may be used in asthma assessment and follow-up.

Patient Education

Education about trigger avoidance, inhaler technique, emergency planning, medication adherence, and safe follow-up.

5. Allergy and Respiratory Testing

5.1 Skin Prick Testing

Skin prick testing is used to look for immediate allergic sensitization to selected allergens. Small drops of allergen extract are placed on the skin and the skin is gently pricked through each drop.

Usual duration: 30 to 45 minutes

Typical result timing: Results are usually read during the same visit

Typical test site: Forearm or upper back

- Patients should tell the clinic about all medicines being taken before the test.
- Some antihistamines can suppress the skin response. The clinic should advise whether and when to stop them.
- Patients should not stop prescription medicine without instruction from a qualified clinician.
- Moisturizer, perfume, or heavy skin cream should not be applied to the planned test area on the day of testing.
- A mild local itchy reaction can occur at test sites and usually settles after the test.

5.2 Patch Testing

Patch testing looks for delayed contact allergy. Small test chambers containing selected substances are applied to the back.

Visit pattern: Application visit, first reading after about 48 hours, and final reading according to the clinician's protocol

Typical total process: Approximately 4 to 7 days

- The test area should be kept dry while patches are attached.
- Heavy exercise and sweating should be avoided while patches are in place.
- Patients should not remove patches unless instructed.
- A positive result must be interpreted together with exposure history; a positive test does not automatically prove the cause of every rash.

5.3 Spirometry

Spirometry measures how much air a patient can blow out and how quickly it can be exhaled. The test may be repeated after a bronchodilator if clinically indicated.

Usual duration: 20 to 30 minutes

Report timing: Preliminary values are available the same day

- Avoid a very heavy meal shortly before testing.
- Avoid strenuous exercise immediately before the test.
- Bring inhalers and a current medication list.
- Only with clinician instruction should any inhaler or respiratory medicine be withheld before the test.

5.4 Serum Specific IgE and Other Laboratory Tests

Blood allergy tests may be requested when they are clinically appropriate. The clinic may collect the sample or issue a laboratory request depending on the test.

Typical result time: 2 to 3 business days for routine specific IgE tests

Complex or send-out tests: May take 5 to 10 business days

- Turnaround time is an estimate and may change if the external laboratory delays a test.
- Results should be interpreted by a clinician in the context of the patient's symptoms and history.

6. Allergen Immunotherapy Safety and Follow-up

Allergen immunotherapy is offered only to selected patients with a compatible clinical history and relevant allergy testing. The treatment plan, dose, route, and duration are individualized by the clinician.

- Patients receiving allergy injections should remain in the clinic observation area for at least 30 minutes after an injection unless the treating clinician requires longer.
- Patients should tell staff if they have fever, active asthma symptoms, recent wheeze, or a previous reaction before receiving the next dose.
- Strenuous exercise should be avoided for approximately 2 hours after an allergy injection.
- The injection site can become mildly red, swollen, or itchy.
- Any breathing difficulty, throat tightness, widespread hives, dizziness, or faintness after treatment requires immediate clinical attention.
- Missed doses may require dose adjustment; patients should not assume the previous dose can simply be repeated.

7. Appointment and Reception Policies

7.1 Booking

- Appointments can be requested through reception, WhatsApp, the clinic website, or the AI assistant.
- A booking is considered confirmed only when the clinic system records the appointment and provides a confirmation or appointment ID.
- Patients should provide a name and reachable phone number for appointment communication.
- For minors, a parent or legal guardian should provide contact details and accompany the child unless the clinic has approved another arrangement.

7.2 Arrival and Late Patients

- Patients should arrive about 10 minutes before the scheduled consultation.
- Patients arriving more than 15 minutes late may need to wait for the next available gap or reschedule.
- Testing visits may require earlier arrival if consent, medication review, or preparation checks are needed.

7.3 Cancellation and Rescheduling

Preferred notice: At least 12 hours before the appointment

- Late cancellation may reduce access to priority rescheduling.
- Patients who cannot attend should cancel rather than simply miss the appointment.
- A cancelled appointment does not guarantee that the same slot will remain available.
- Rescheduling is subject to the current live schedule.

7.4 Follow-up Visits

- Follow-up timing depends on the patient's condition, test results, and treatment plan.
- Patients should bring relevant reports, medicine lists, inhalers, and previous allergy test records.
- If symptoms have changed significantly since the last visit, the patient should mention the change at booking or check-in.

8. Fees, Payments, and Receipts

The following amounts are demo values for RAG testing. In production, consultation charges should be verified against the clinic billing or doctor database.

Dr. Hamid Bashir consultation: Rs. 2,500

Dr. Sana Bashir consultation: Rs. 2,200

Dr. Areeba Khan consultation: Rs. 2,000

Skin prick test: From Rs. 4,000 depending on the selected allergen panel

Patch test: From Rs. 6,500 depending on the selected series

Spirometry: Rs. 1,500

Allergy injection administration: Rs. 700 per clinic-administered visit, excluding allergen product cost

- Accepted payment methods: cash, debit/credit card, bank transfer, and supported digital wallet.
- A printed or electronic receipt can be requested.
- The clinic does not provide direct insurance billing in this demo policy.
- Patients who need reimbursement documents should request a detailed receipt from reception.

9. Reports and Medical Records

- Skin prick test results are generally discussed during the same visit.
- Spirometry values are generally available the same day, but formal interpretation may be completed during the consultation.
- Routine specific IgE results usually take 2 to 3 business days.

- Patch testing requires multiple readings and final interpretation may take 4 to 7 days.
- Reports can be collected from reception or shared electronically when the clinic has verified the patient's contact details.
- Medical records are not released to an unrelated person without appropriate patient authorization.
- Patients requesting copies of older records should provide identification details that allow the clinic to locate the correct file.

10. Emergency and Safety Guidance

Bashir Clinic & Allergy Center is an outpatient clinic and is not a full emergency department.

Severe breathing difficulty, rapidly worsening wheeze, swelling of the tongue or throat, fainting, collapse, confusion, blue lips, or symptoms suggesting anaphylaxis require emergency medical care. A patient with severe symptoms should use their prescribed emergency plan if one exists and seek the nearest emergency service immediately.

- Do not wait for a routine clinic appointment when severe symptoms are present.
- Do not use the AI assistant as a substitute for emergency assessment.
- The clinic may advise transfer to an emergency department when symptoms exceed outpatient capability.

11. Teleconsultation

- Teleconsultation is mainly intended for selected follow-up visits and review of stable symptoms or reports.
- First-time severe allergic reactions, suspected anaphylaxis, procedures, skin testing, patch testing, and spirometry require in-person assessment.
- A teleconsultation may be converted to an in-person visit if examination or testing is required.
- Patients should upload or send relevant reports only through clinic-approved channels.

12. Medication and Prescription Policy

- The clinic does not recommend stopping prescribed medicine without clinician advice.
- Prescription refills may require review when the medicine is high risk, the patient's condition has changed, or the previous consultation is no longer recent.
- The AI assistant must not prescribe a new medicine, change a dose, or instruct a patient to stop medicine.
- Patients should report previous drug reactions before receiving or being prescribed a new medicine.
- Patients should bring a current medication list to allergy and asthma consultations.

13. Privacy and Communication

- Patient information should be used only for care, scheduling, billing, and authorized clinic operations.
- The clinic should verify identity before sharing sensitive appointment or medical information.
- Patients should avoid sending unnecessary sensitive information in public or shared messaging channels.
- Clinical reports should be shared only through approved channels after the recipient is verified.
- The AI assistant should never expose another patient's details.

14. Frequently Asked Questions

Q: Do I need an appointment?

A: Appointments are recommended. Walk-in patients may be seen only if a clinician has capacity, and waiting time cannot be guaranteed.

Q: Can I come for an allergy test without seeing a doctor?

A: Some repeat or pre-planned tests may be performed directly, but first-time testing usually requires clinical review so the correct allergens and safest test method can be selected.

Q: How long does skin prick testing take?

A: A typical skin prick testing visit takes about 30 to 45 minutes, and the result is usually read during the same visit.

Q: Should I stop antihistamines before skin testing?

A: Some antihistamines can interfere with skin test results. The clinic should review the exact medicine and tell the patient whether it should be withheld. Patients should not stop prescription medicine on their own.

Q: Can allergy testing be done while I have fever or an asthma flare?

A: The clinician may postpone some tests or immunotherapy when a patient has fever, active wheeze, or unstable asthma. The patient should tell the clinic before coming.

Q: How soon are blood allergy test results ready?

A: Routine serum specific IgE results are usually available in 2 to 3 business days. Some send-out tests may take 5 to 10 business days.

Q: How long does patch testing take?

A: Patch testing is a multi-visit process. Patches are usually applied first, read after about 48 hours, and reviewed again according to the clinician's protocol. Final interpretation commonly takes 4 to 7 days.

Q: Can I shower during patch testing?

A: The patch test area should be kept dry while the patches are attached. The patient should follow the clinic's instructions about bathing during the testing period.

Q: Is allergy immunotherapy a one-time treatment?

A: No. Immunotherapy is a planned course of treatment that may continue for months or years depending on the patient, allergen, response, and treatment method.

Q: Why do I have to wait after an allergy injection?

A: Patients are observed because a systemic allergic reaction can rarely occur after an injection. The demo clinic policy requires at least 30 minutes of observation unless the clinician specifies longer.

Q: Can I exercise after an allergy injection?

A: Strenuous exercise should be avoided for approximately 2 hours after an allergy injection because exercise can affect reaction risk.

Q: Can children be tested for allergy?

A: Children can be evaluated for allergy. The choice of test depends on age, symptoms, history, skin condition, medicines, and the clinical question.

Q: Does a positive allergy test always mean I am allergic?

A: No. A positive test shows sensitization and must be interpreted together with the patient's actual symptoms and exposure history.

Q: Can the clinic diagnose food intolerance with a broad panel?

A: The clinic uses history-guided evaluation. Broad testing without a clinical reason can produce misleading positive results and is not a substitute for medical assessment.

Q: Can I receive emergency treatment at the clinic?

A: The clinic can respond to emergencies that occur during a clinic procedure, but it is not a full emergency department. Severe symptoms outside a clinic procedure require urgent emergency medical care.

Q: Can reports be sent on WhatsApp?

A: Reports may be shared electronically only through clinic-approved channels after contact details and recipient identity are verified.

Q: Can someone else collect my report?

A: The clinic may require patient authorization and identification before releasing a report to another person.

Q: Can I book through the AI assistant?

A: Yes, if the live clinic system supports the requested doctor, date, and time. A booking is confirmed only after the backend records it and returns a confirmation or appointment ID.

Q: Can the AI assistant tell me which medicine to take?

A: No. The AI assistant may provide general health education but should not prescribe medicine, change doses, or create a personal treatment plan.

Q: What if the answer is not in the clinic records?

A: The assistant should say that the information could not be verified and should direct the patient to reception or a clinician rather than inventing an answer.

15. RAG Retrieval Notes for the AI System

This section defines how the assistant should use this document. It is intended for developers and clinic administrators.

- Use semantic retrieval rather than language-specific phrase lists.
- A patient may ask in English, Urdu, Roman Urdu, mixed language, or with spelling mistakes; retrieval should rely on meaning.
- Rerank retrieved passages against the exact patient question.
- If no retrieved passage contains sufficient evidence, return no verified document answer.
- Do not combine a weakly related passage with general model knowledge to create a clinic-specific fact.
- For live appointment slots, appointment status, patient records, and current doctor availability, use the backend database.
- For clinic policies, test preparation, service descriptions, report timelines, and static instructions, use RAG evidence.
- If a static document conflicts with a live backend record for an operational fact, the live backend record takes priority.
- Store source document name, section or semantic title, passage index, and source text with each retrieved result.
- Version and effective date should be preserved so stale information can be identified.

16. Document Change Log

Version 1.0: Initial synthetic RAG test knowledge base created in September 2026

Before production deployment, clinic administration should replace all demo contact information, fees, schedules, service prices, and policies with approved values. The demo warning on the first page should then be removed.